

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Levonorgestrel 1.5mg tablets (Levonelle) and Ulipristal acetate 30mg tablets (ellaOne) for the treatment of Emergency Contraception

Summary of NICE / NICE CKS Guidelines for Emergency Contraception

Overview

Emergency contraception is used to prevent pregnancy after unprotected sexual intercourse or contraceptive failure.

Assessment

The following assessment factors should be considered:

- Time since unprotected sexual intercourse (UPSI)
- Previous UPSI in the cycle
- Current contraception use
- Date of last menstrual period
- Regular cycle length
- Potential drug interactions (enzyme inducers)
- BMI and weight
- Breastfeeding status

Management

- **Levonorgestrel:** Within 72 hours (3 days) of UPSI; most effective within 12 hours
- **Ulipristal acetate:** Within 120 hours (5 days) of UPSI
- Consider copper IUD as most effective emergency contraception option (refer to GP/clinic)

Red flags

- Suspected existing pregnancy
- Severe asthma (ulipristal)
- Repeated emergency contraception requests (signpost to ongoing contraception)

Patient Group Direction

for the supply/administration of

Levonorgestrel 1.5mg tablet (Levonelle)

for the treatment of

Emergency Contraception

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Emergency contraception within 72 hours of unprotected sexual intercourse
Inclusion criteria	Females of reproductive age (typically 11-55 years) Within 72 hours of unprotected sexual intercourse (UPSI) or contraceptive failure Valid informed consent obtained No known or suspected pregnancy
Exclusion criteria	Known or suspected pregnancy Hypersensitivity to levonorgestrel or any component of the formulation Severe hepatic impairment (e.g., cirrhosis) Hereditary galactose intolerance Acute intermittent porphyria
Cautions	Current or recent use of enzyme-inducing drugs (anticonvulsants, antiretrovirals, St John's Wort) - efficacy may be reduced; consider double dose (3mg) or use ulipristal as alternative Malabsorption conditions affecting drug absorption (e.g., Crohn's

	disease) History of ectopic pregnancy Breastfeeding - avoid for 8 hours after dose Weight $\geq 70\text{kg}$ or BMI ≥ 26 - consider double dose (3mg) or ulipristal as alternative Recent unprotected intercourse where pregnancy risk is higher (e.g., mid-cycle)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Levonorgestrel 1.5mg tablet (Levonelle)
Legal category	POM (Prescription-only medicine when supplied under PGD)
Storage	Store below 25°C in a dry place. Keep in original packaging.
Route/method of administration	Oral - swallow tablet whole with water
Dose and frequency	Standard dose: 1.5mg as a single dose, taken as soon as possible after UPSI (ideally within 12 hours, but effective up to 72 hours) Double dose: 3mg may be considered if weight $\geq 70\text{kg}$, BMI ≥ 26 , or taking enzyme-inducing drugs
Quantity to be administered and/or supplied	1 tablet (1.5mg single treatment) or 2 tablets (3mg if double dose indicated)
Maximum or minimum treatment period	Single dose only; repeated use not recommended within the same cycle
Adverse effects	Nausea and vomiting (if vomiting occurs within 2 hours of taking the tablet, re-dosing should be considered) Abdominal pain and abdominal discomfort Headache Fatigue and tiredness Dizziness and vertigo Breast tenderness Menstrual irregularity (period may be delayed or early) Mood changes (rare) Pelvic pain

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	If vomiting occurs within 2 hours of taking the tablet, contact your healthcare provider for advice as re-dosing may be needed. Your next period may be earlier or later than expected. If your period is more than 7 days late, perform a pregnancy test or contact your doctor. Use barrier contraception (condoms) until your next period to reduce risk of pregnancy. Consider starting or continuing regular contraception (after waiting 5 days if you have taken ulipristal) - discuss options with your doctor or healthcare provider. This emergency contraception does not protect against sexually transmitted infections (STIs). Discuss STI testing if appropriate. If you experience any adverse effects or have concerns, contact your doctor or healthcare provider. Remember that emergency contraception works best when taken as soon as possible after unprotected sexual intercourse.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017
<https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017
<https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Emergency Contraception
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
	(GPhC: 2046322)		
Name of professional group signatory	Job title & name of organisation	Signature	Date
	(GPhC: 2046322)		

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction
for the supply/administration of
Ulipristal acetate 30mg tablet (ellaOne)
for the treatment of
Emergency Contraception

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Emergency contraception within 120 hours of unprotected sexual intercourse
Inclusion criteria	Females of reproductive age (typically 11-55 years) Within 120 hours (5 days) of unprotected sexual intercourse (UPSI) or contraceptive failure Valid informed consent obtained No known or suspected pregnancy
Exclusion criteria	Known or suspected pregnancy Hypersensitivity to ulipristal acetate or any component of the formulation

	Severe hepatic impairment (e.g., cirrhosis) Severe asthma insufficiently controlled by oral glucocorticoids Hereditary galactose intolerance Concurrent or recent use of hormonal contraceptives (hormonal contraception must not be started until 5 days after taking ulipristal)
Cautions	Current or recent use of enzyme-inducing drugs (anticonvulsants, antiretrovirals, St John's Wort) - efficacy may be reduced; consider copper IUD as alternative Malabsorption conditions affecting drug absorption History of ectopic pregnancy Breastfeeding - avoid breastfeeding for 7 days after dose Repeated use in the same cycle not recommended Hormonal contraception should not be started until 5 days after taking ulipristal
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Ulipristal acetate 30mg tablet (ellaOne)
Legal category	POM (Prescription-only medicine)
Storage	Store below 25°C. Keep in original packaging.
Route/method of administration	Oral - swallow tablet whole with water
Dose and frequency	30mg as a single dose, taken as soon as possible after UPSI (effective up to 120 hours/5 days)
Quantity to be administered and/or supplied	1 tablet (30mg single treatment)
Maximum or minimum treatment period	Single dose only; repeated use not recommended within the same cycle
Adverse effects	Headache Nausea Abdominal pain and abdominal discomfort Dysmenorrhoea (painful period) Fatigue and tiredness Dizziness and vertigo

	Mood changes and mood swings Myalgia (muscle pain) Back pain Breast tenderness Menstrual irregularity Pelvic pain
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
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	This emergency contraception does not protect against sexually transmitted infections (STIs). Discuss STI testing if appropriate. If you experience any adverse effects or have concerns, contact your doctor or healthcare provider. Remember that emergency contraception works best when taken as soon as possible after unprotected sexual intercourse.
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